

Home



Manchester GPs' Reviews

424 GP surgeries profiled.

3,021,833 patients across 360 practices.

SCORE SOURCE

Google

GP
Survey

Gap

Google data here is from this repo's merged review collection.

AREA OVERLAYS

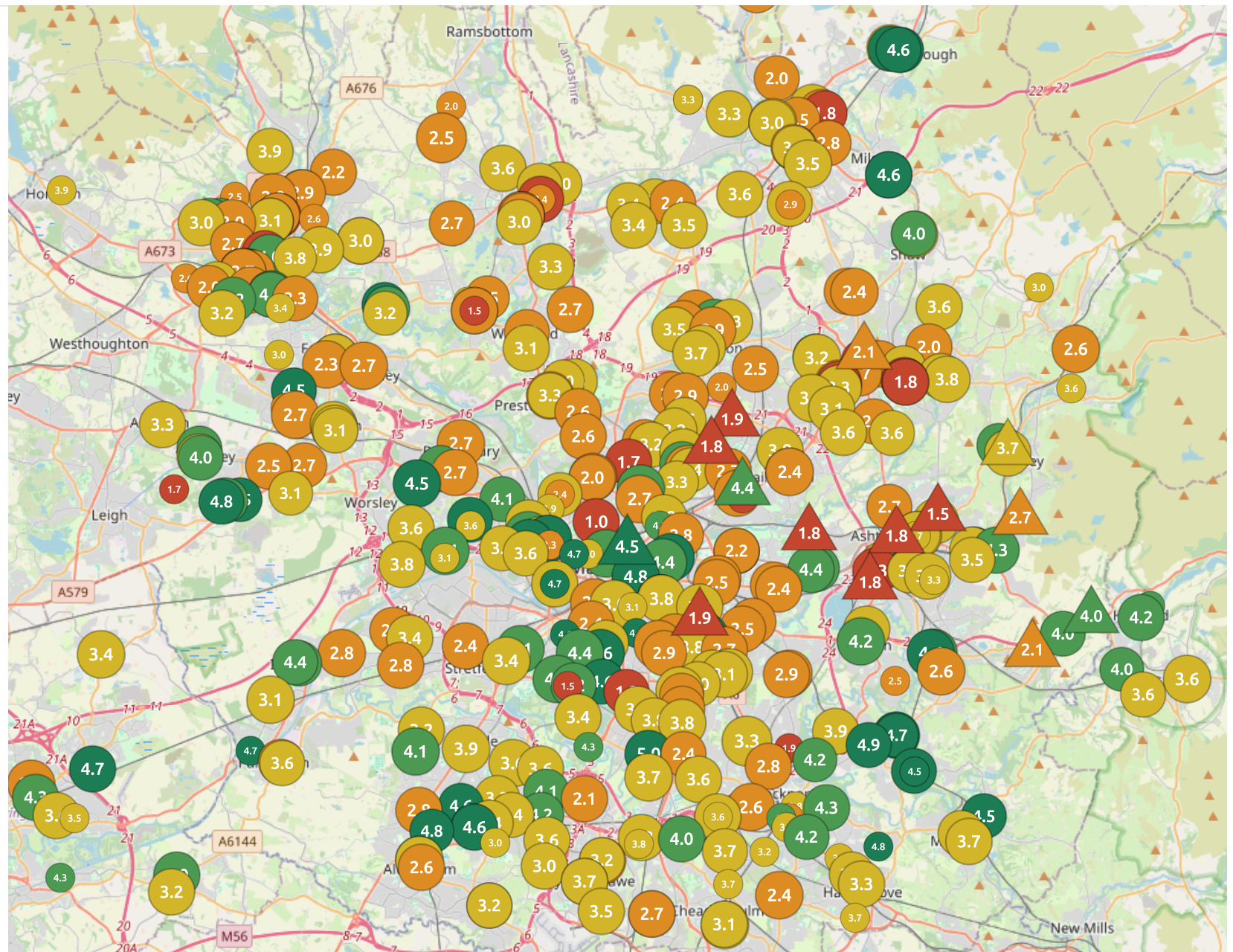
☐ Est.
population

☐ Deprivation

MANAGEMENT

GTD stays on as the baseline. Add up to 4 more management companies to compare their average google rating with GTD.

- ☐ Caritas GP Partnership 3.40 avg · 2 ▲
- ☐ Cheetham Hill Medical Centre 3.45 avg · 2 ●
- ☐ Clarendon Medical Practice - Pendleton 3.40 avg · 2 ▼



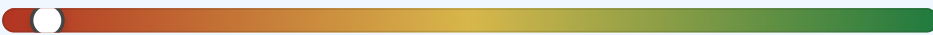
Quick comparisons - Showing Reviews

New Bank and GTD always stay visible here. Click a practice on the map to compare it with New Bank. Use the management tickboxes to compare other companies with GTD. “Nearby” means within 2.5 miles.

New Bank Health

Baseline practice · M12 4JE

On Google rating, New Bank Health is 1.2 below than the nearby average typical score and 1.4 below than the the wider map average typical score. It sits around the 6th percentile on this map.



Worse than **397** practices

Better than **20** practices

Current score

1.9

This practice · 1
usable value

3.1

Nearby average ·
47 peers

3.3

Wider map
average · 423
peers

Nearby: 1.2
below. Wider
map: 1.4 below.

Survey reply rate

10%

This practice

14%

Nearby average ·
47 peers

24%

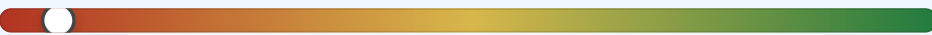
Wider map
average · 423
peers

Reply rate sits
around the 4th
percentile on this
map.

GTD Healthcare

Baseline management company · 14 practices

On Google rating, GTD Healthcare is 0.7 below than the nearby non-company average typical score and 0.7 below than the the wider map average typical score. It sits around the 6th percentile against the other management companies on this map.



Worse than **15** management companies

Better than **1** management company

Current average

2.6

Company average
· 14 practices
with data

3.3

Nearby others ·
156 peers

3.3

Wider map
average · 410
peers

Nearby others:
0.7 below. Wider
map: 0.7 below.

Survey reply rate

19%

Company average
· 14 practices
with data

20%

Nearby others ·
156 peers

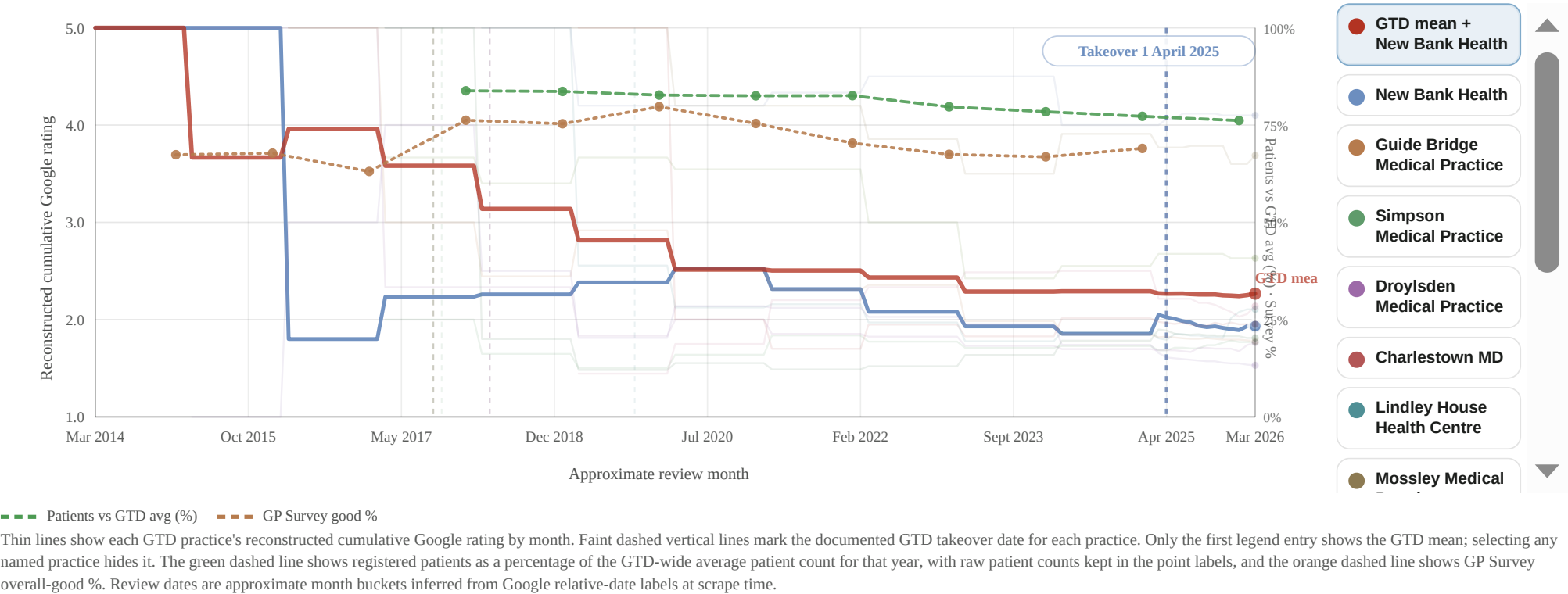
24%

Wider map
average · 410
peers

Reply rate sits
around the 22th
percentile on this
map.

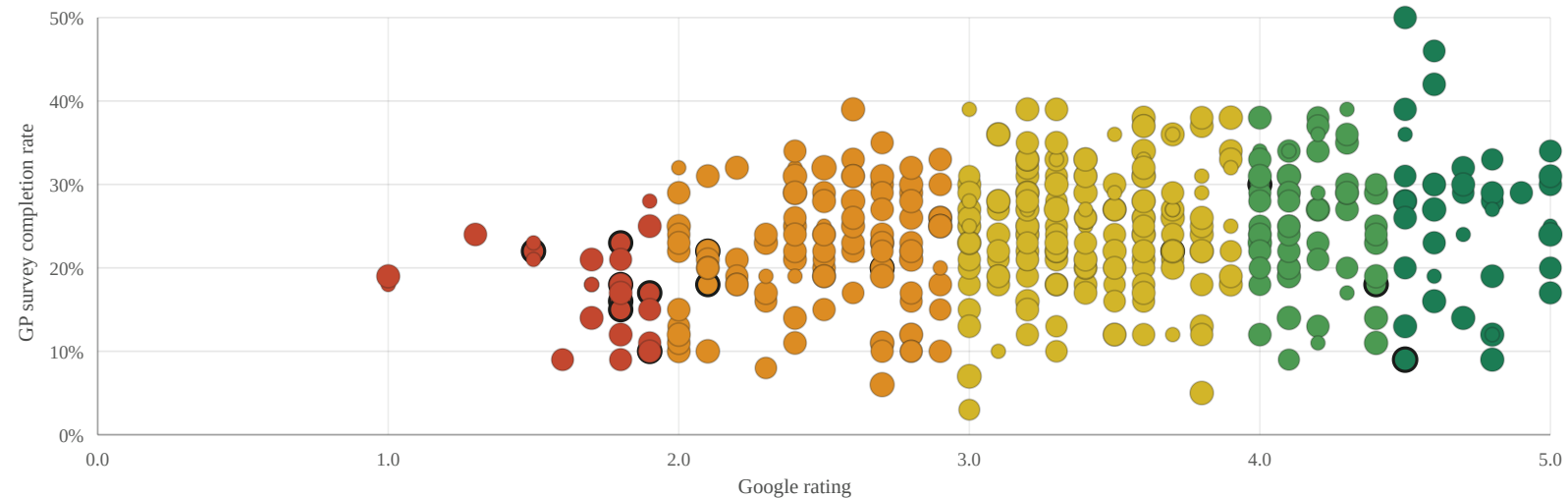
GTD Score Over Time - Showing Reviews

12 of 13 GTD practices contribute to this chart, based on 952 parsed Google review dates and ratings. Thin lines are practice-level reconstructed cumulative averages, dashed vertical lines mark documented GTD takeover dates, and the bold line is the mean of available practice trajectories. Relative dates are anchored to the scrape file timestamp 2026-03-10. Default view shows the GTD mean with New Bank Health as the reference track. Green dashed: registered patients as a share of the GTD yearly average, with raw patient counts left in the point labels. Orange dashed: GP Survey good %. 1 GTD practice still have no usable dated review history in the scrape: City Health Centre.



Completion Rate vs Score - Showing Reviews

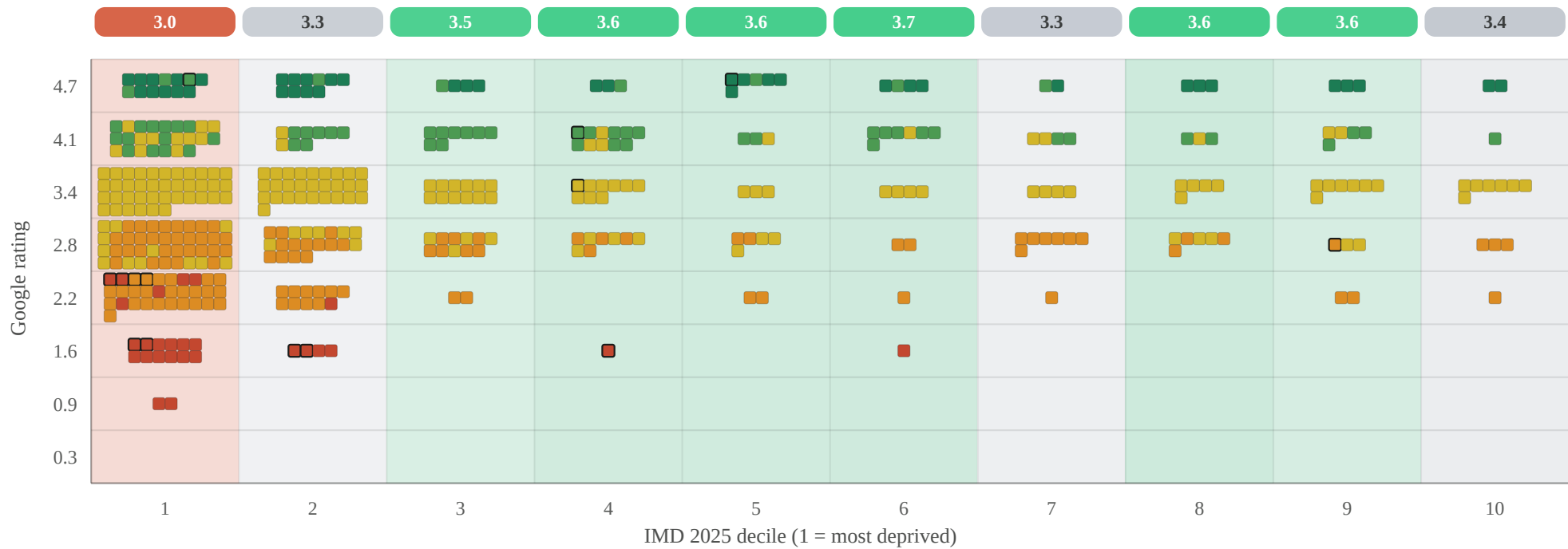
424 practices have both GP survey completion data and a usable google rating value. Median completion is 24%. Pearson r is 0.25. New Bank Health is at 10% completion and sits around the 5th percentile for completion in this set.



Y-axis is GP Patient Survey completion rate. X-axis changes with the selected score source.

Score vs Deprivation - Showing Reviews

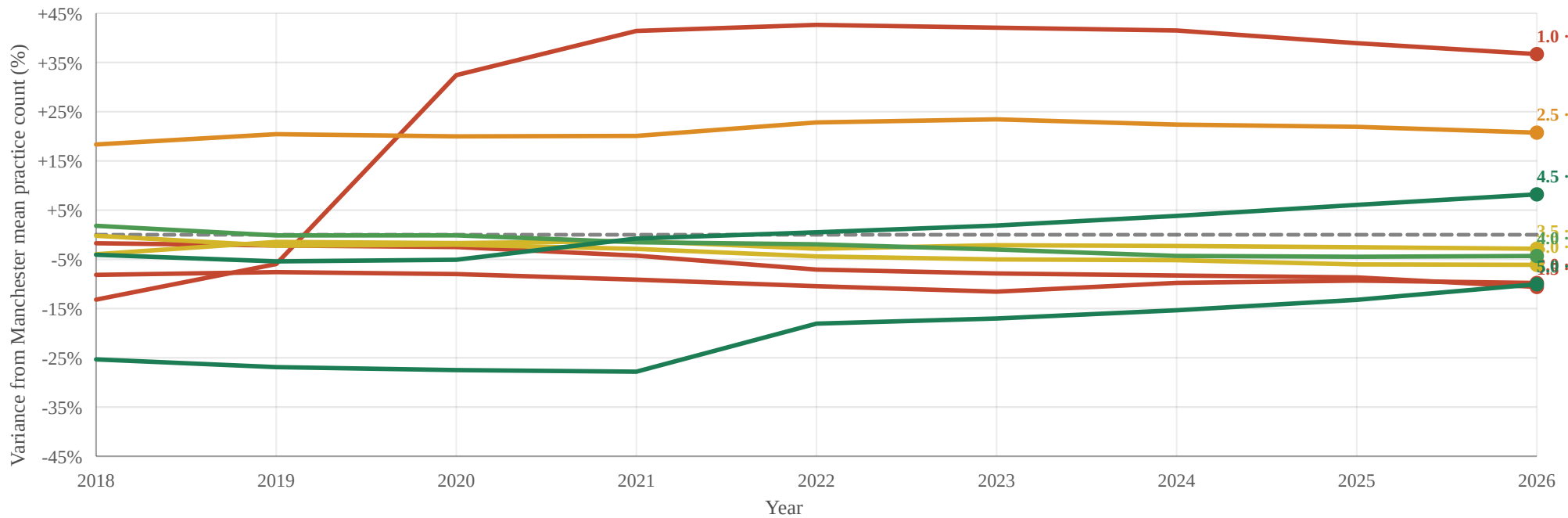
424 practices have both a usable google rating value and mapped IMD 2025 decile. Median decile is 2 and median google rating is 3.3. Pearson r for score vs decile is 0.21. Column tint shows whether each decile skews better (green), worse (red), or similar (grey) versus the overall distribution, with stronger colour meaning a more different distribution. Strongest departure is decile 8.



X-axis is IMD 2025 decile (1 = most deprived). Y-axis changes with the selected score source, including the signed survey/Google gap.

Registered Patients Over Time - Coloured by Reviews

360 practices have multi-year patient-count histories in this chart. Coloured lines show the average trajectory for each current reviews band, and the dashed grey line is the Manchester average practice count for each year. With Flatten Global on, the y-axis shows deviation from that yearly Manchester mean, so the chart shows which score bands are gaining or losing relative share rather than absolute patient volume. New Bank shifts from -16.2% to -23.8% versus the Manchester mean practice count.



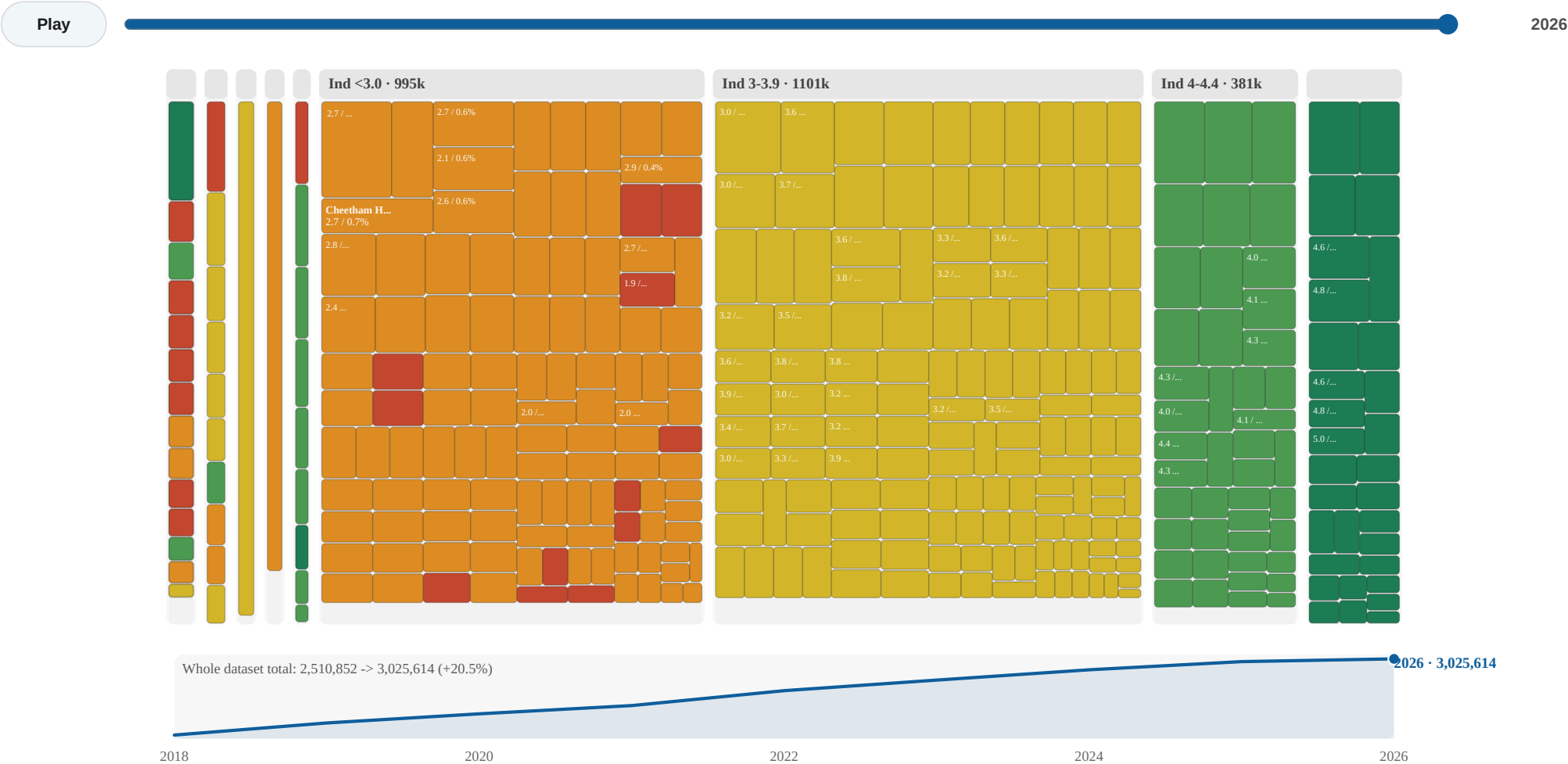
X-axis is year. Y-axis is registered patients. Thin lines show practice list-size trajectories, coloured by the current selected score; the dashed grey line is the Manchester-wide average practice count for each year.

Patient growth footnote: both stronger and weaker score bands show only VERY slight divergence from the mean, with better-rated / better-surveyed practices gaining patients a little faster and worse-performing GPs gaining a little more slowly. That suggests patients ARE moving toward better doctors, but INCREDIBLY slowly. One possible reading is that poor access can still trap patients who need more access, convenience, or support most, while others who have a choice (and know it) are more able to switch. This is however in a context of relative to growth, where population pressure is present, but seemingly not a large driver of experience. This suggests policy drives experience more than population pressure, though further investigation might help here.

Patient Count Treemap - Coloured by Reviews

☒ Flatten for Population ☒

360 practices are shown for 2026 across 9 treemap columns. Rectangle area uses a fixed scale of 9.52 normalised patients per pixel, so the same area basis is used across all years and groups. Named operator columns stay separate, while independent/other practices are split by Google review band. Colour shows current reviews, and labels use score / dataset share. Largest live block this year is Independent / other · review 3.0-3.9 with 1101092 normalised patients, while the widest reserved column is GTD Healthcare at peak 80970 normalised patients. Whole-dataset registered patients move from 2,510,852 to 3,025,614 across this series (+20.5%). In this mode each year is rescaled to the 2026 total, so box changes reflect redistribution within the pool rather than overall pool growth.



This uses a fixed-scale grouped strip-treemap rather than re-squarifying each frame, so practice blocks mostly grow and shrink in place. Block area is registered patients in the chosen year, on the same patient-to-pixel scale for all years; colour and score label use the currently selected metric. Independent / other is split into Google review-score bands so better and worse destinations can be compared. The small strip-chart below shows the whole-dataset registered-patient total over the same years.

Conclusions

This page suggests GTD is the weakest-performing management group in this catchment, with New Bank sitting at or near the bottom even within the deprived groups it belongs to. On both public reviews and GP Patient Survey measures, GTD has too many poor-performing practices relative to the wider sample.

The Google-versus-survey gap matters because GTD practices often show a larger mismatch than typical surgeries, while survey return rates are low enough to leave room for hidden dissatisfaction. New Bank looks off-curve rather than merely unlucky within the normal local range.

The deprivation views also point to a real but limited dataset-wide lean: more deprived areas do tend to have somewhat worse review and survey distributions. That said, this page does not suggest that the whole NHS only fails poorer areas or that poor areas only contain poor-quality practices (it's surprisingly a little more evenly distributed than I expected). Wider regional or national sampling could test how much of that lean is structural versus local.

The practices sampled here also saw total registered patients rise by about 514,762 between 2018 and 2026, roughly 20.5% growth across the period. That is a significant resource-pressure context in its own right, so this overall pattern suggests a region that in many places responded surprisingly well to new demand, especially through years of reduced funding: some practices did not just maintain patient-facing scores, but improved them.

The practical conclusion is local rather than fatalistic. National context matters, but the strongest actionable result here is that GTD, and especially New Bank, are performing worse than most of the sample even after allowing for deprivation.

Change across GTD is both plausible and necessary, and in more deprived areas the benefit of improvement is larger because easy access matters most where health need is greatest.